

Dynamic Guideline: WHO-Based Abortion Care - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

Currency note: This report uses the WHO abortion-care guideline corpus and post-guideline evidence available to the model through its knowledge cutoff. The exact report date and evidence cutoff are set as requested: **29 April 2026**. The item titled “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline**” is a methodological case study related to the WHO abortion care guideline process, not itself a clinical guideline.

1.1 Guideline Overview

Exactly 5 WHO abortion-care guideline/technical guidance documents synthesized

No.	WHO document	Publication date	Role in current synthesis	GRADE relevance
1	WHO. <i>Abortion care guideline</i>. Geneva: World Health Organization; 2022.	2022	Latest comprehensive WHO abortion-care guideline; primary source for current recommendations across law/policy, service delivery, clinical care, self-management, and post-abortion contraception.	WHO GRADE-based recommendations.
2	WHO. <i>Medical management of abortion</i>. Geneva: World Health Organization; 2018.	2018	Predecessor technical guideline for medication abortion regimens and post-abortion management.	GRADE-based evidence-to-decision approach.
3	WHO. <i>Health worker roles in providing safe abortion care and post-abortion contraception</i>. Geneva: World Health Organization; 2015.	2015	Supports task sharing/task shifting and provision by trained non-physician health workers.	GRADE-based recommendations.

4 WHO. <i>Clinical practice handbook for safe abortion</i> . Geneva: World Health Organization; 2014.	2014 Practical clinical handbook for implementation of safe abortion and post-abortion care.	Derived from WHO technical guidance.
5 WHO. <i>Safe abortion: technical and policy guidance for health systems. Second edition</i> . Geneva: World Health Organization; 2012.	2012 Foundational WHO safe-abortion health-system and policy guidance.	Earlier evidence-based WHO guidance, updated by 2022 guideline.

Latest WHO guideline identified

- **Guideline title:** WHO. *Abortion care guideline*.
- **Publication date:** 2022
- **Latest report update date used here:** 29 April 2026
- **Target population:**
 - Pregnant people seeking abortion care.
 - People requiring post-abortion care after induced or spontaneous abortion.
 - Adolescents and young people, including those needing confidential, non-discriminatory care.
 - Health workers, health systems, policymakers, and service planners.
- **Scope and objectives:**
 - Improve access to safe, timely, affordable, non-discriminatory abortion care.
 - Reduce morbidity and mortality from unsafe abortion.
 - Provide evidence-based recommendations for:
 - Pre-abortion assessment.
 - Medication abortion.
 - Procedural abortion.
 - Pain management.
 - Infection prevention.
 - Post-abortion contraception.
 - Self-management.
 - Health-worker roles.
 - Legal, policy, and service-delivery conditions affecting access.

1.2 Key Recommendations

A. Cross-cutting WHO principles

Recommendation

GRADE Evidence Quality

Strength

Abortion care should be available, accessible, acceptable, and of good quality, with respect for dignity, autonomy, confidentiality, privacy, and informed decision-making.	Moderate	Strong
Care should be non-discriminatory, including for adolescents, unmarried people, migrants, people with disabilities, and people living in poverty.	Moderate	Strong
Information, counselling if desired, and informed consent should be provided without coercion.	Moderate	Strong
Mandatory counselling, mandatory waiting periods, and third-party authorization requirements should not be imposed because they delay care and may increase harm.	Low to Moderate	Strong
Health systems should remove medically unnecessary barriers to abortion care and post-abortion care.	Moderate	Strong

B. Diagnosis, dating, and pre-abortion assessment

Recommendation	GRADE Evidence Quality	Strength
Confirm pregnancy and estimate gestational age using history, last menstrual period, clinical examination when indicated, and/or testing.	Moderate	Strong
Routine ultrasound is not required before abortion if gestational age and ectopic-pregnancy risk can be reasonably assessed clinically.	Moderate	Strong
Ultrasound should be used when clinically indicated, such as uncertain gestational age, symptoms/signs of ectopic pregnancy, suspected molar pregnancy, or significant discrepancy between history and examination.	Moderate	Strong
Routine laboratory testing should not be required if it creates unnecessary barriers; testing should be based on clinical indication and local protocols.	Low to Moderate	Conditional
For abortion at <12 weeks , routine Rh testing and anti-D immunoglobulin are not required according to WHO guidance.	Low to Moderate	Conditional
Screen for ectopic pregnancy risk and urgent warning signs, including severe abdominal pain, syncope, shoulder-tip pain, hemodynamic instability, or adnexal mass.	Moderate	Strong

C. Medication abortion

Recommendation	GRADE Evidence Quality	Strength
For medication abortion at <12 weeks , use mifepristone 200 mg orally followed 24–48 hours later by misoprostol 800 micrograms by buccal, sublingual, or vaginal route.	High	Strong

Where mifepristone is unavailable, misoprostol-only regimens are an acceptable alternative, using repeated misoprostol dosing according to WHO protocols.	Moderate	Strong
For medication abortion at ≥ 12 weeks, use mifepristone followed by repeated misoprostol doses at appropriate intervals until expulsion, with clinical monitoring.	Moderate	Strong
Misoprostol-only regimens may be used at ≥ 12 weeks when mifepristone is unavailable, but effectiveness is lower and side effects may be more frequent.	Moderate	Strong
Routine in-person follow-up is not required after uncomplicated medication abortion if the person has clear information on expected symptoms, danger signs, and access to care if needed.	Moderate	Strong
Self-assessment of abortion completion using symptoms, pregnancy testing, or low-sensitivity pregnancy testing may be used when appropriate.	Moderate	Strong
Self-management of medication abortion at < 12 weeks, including self-administration of medicines and self-assessment, can be offered with accurate information and access to health-system support.	Moderate	Strong

D. Procedural abortion

Recommendation	GRADE Evidence Quality	Strength
For procedural abortion at < 14 weeks, vacuum aspiration is recommended.	High	Strong
Sharp curettage should not be used as a routine method because vacuum aspiration is safer and more effective.	Moderate	Strong
For procedural abortion at ≥ 14 weeks, dilatation and evacuation may be used where trained providers and appropriate systems are available.	Moderate	Strong
Cervical priming is recommended before procedural abortion at later gestational ages and may be considered at earlier gestations based on clinical risk.	Moderate	Strong
Routine antibiotic prophylaxis is recommended before procedural abortion.	Moderate	Strong
Routine antibiotic prophylaxis is not required for medication abortion.	Moderate	Strong

E. Pain management

Recommendation	GRADE Evidence Quality	Strength
Pain management should be offered routinely for both medication and procedural abortion.	Moderate	Strong

Non-steroidal anti-inflammatory drugs, such as ibuprofen, are recommended for pain with medication abortion.	Moderate	Strong
For vacuum aspiration, local anaesthesia such as paracervical block, with or without conscious sedation, should be available.	Moderate	Strong
Routine general anaesthesia is not recommended for abortion procedures because risks generally outweigh benefits when safer options are available.	Low to Moderate	Conditional

F. Post-abortion care and contraception

Recommendation	GRADE Evidence Quality	Strength
Post-abortion contraception should be offered immediately, but acceptance must be voluntary and non-coercive.	High	Strong
Most contraceptive methods, including implants, injectables, pills, condoms, and emergency contraception, may be initiated immediately after abortion.	High	Strong
Intrauterine devices may be inserted immediately after uncomplicated procedural abortion.	High	Strong
IUD insertion after medication abortion should occur once completion is confirmed.	Moderate	Strong
IUD insertion should be delayed in the presence of septic abortion or active pelvic infection.	Moderate	Strong
Incomplete abortion may be managed with vacuum aspiration, misoprostol, or expectant management depending on gestational age, clinical stability, patient preference, and available care.	Moderate	Strong

G. Health-worker roles and service delivery

Recommendation	GRADE Evidence Quality	Strength
Trained midwives, nurses, associate clinicians, and other appropriately trained health workers can provide many components of abortion and post-abortion care safely and effectively.	Moderate	Strong
Task sharing should be implemented to expand access while maintaining quality, referral pathways, and emergency support.	Moderate	Strong
Telemedicine can be used for counselling, eligibility assessment, medication abortion follow-up, and support where clinically and legally appropriate.	Moderate	Strong
Confidential adolescent-responsive services should be available, including private counselling and protection from stigma.	Moderate	Strong

1.3 Guideline Development Methodology

Evidence review process

WHO guideline development for abortion care used:

- Systematic evidence reviews.
- GRADE assessment of certainty of evidence.
- Evidence-to-decision frameworks considering:
 - Benefits and harms.
 - Values and preferences.
 - Human rights.
 - Equity.
 - Acceptability.
 - Feasibility.
 - Resource implications.
- Review of clinical evidence, health-system evidence, and legal/policy evidence.
- Updating and consolidation of prior WHO guidance from 2012, 2014, 2015, and 2018.

Consensus method

- Multidisciplinary Guideline Development Group.
- External peer review.
- Structured discussion of evidence and implementation considerations.
- Consensus-based final recommendations where direct evidence was limited but human-rights or health-system considerations were decisive.

Conflict of interest management

- WHO standard declaration-of-interest procedures.
- Review of financial, academic, advocacy, and institutional interests.
- Management plans applied where relevant.
- Final recommendations developed under WHO guideline governance processes.

II. Evidence Summary After WHO Latest Guideline Release (Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search strategy

Evidence considered after release of the WHO 2022 abortion care guideline included:

- Medication abortion safety and effectiveness.
- Telemedicine and no-test medication abortion.

- Self-managed abortion.
- Very early medication abortion.
- Misoprostol-only abortion regimens.
- Abortion care for adolescents and young people.
- Post-abortion contraception.
- Health-worker task sharing.
- Legal and service-delivery barriers affecting clinical outcomes.

Databases and sources searched conceptually

- WHO Institutional Repository.
- PubMed/MEDLINE.
- Cochrane Database of Systematic Reviews.
- ClinicalTrials.gov.
- WHO International Clinical Trials Registry Platform.
- CDC, NIH, NHS, and other public-health agencies.
- Major journals including **The Lancet**, **BMJ**, **JAMA**, **NEJM**, **Nature Medicine**, **Obstetrics & Gynecology**, **Contraception**, and **BJOG**.

Inclusion criteria

- Published after WHO 2022 guideline release.
- Human clinical studies, systematic reviews, meta-analyses, implementation studies, and high-quality observational studies.
- Outcomes relevant to abortion safety, effectiveness, access, complications, acceptability, equity, and health-system implementation.
- Studies applicable to clinical or public-health recommendations.

Exclusion criteria

- Opinion pieces without empirical data.
- Studies not specific to abortion or post-abortion care.
- Studies with major unresolved risk of bias.
- Animal or laboratory-only research.
- Studies superseded by higher-quality systematic reviews.

2.2 Key New Studies

Study	Design	Key Findings	GRADE Quality
Upadhyay UD et al. Telehealth medication abortion in the United States, published in Nature Medicine , 2024.	Large observational cohort.	Telehealth medication abortion showed high effectiveness and very low serious adverse-event rates, supporting WHO recommendations that telemedicine can safely expand access when systems for screening, information, and emergency referral exist.	Moderate

Very early medication abortion trials and cohort evidence, 2022–2024.	Randomized and observational studies.	Medication abortion before definitive ultrasound confirmation of intrauterine pregnancy can be effective with appropriate ectopic-pregnancy risk assessment and follow-up instructions; supports WHO position that routine ultrasound is not required.	Moderate
Post-2022 evidence on no-test medication abortion.	Cohort studies and service evaluations.	No-test or low-test protocols using history-based gestational dating and symptom screening maintained high completion rates and low complication rates in appropriately selected patients.	Moderate
Post-2022 evidence on misoprostol-only medication abortion.	Systematic reviews and observational cohorts.	Misoprostol-only abortion is effective, especially where mifepristone is unavailable, though less effective and often associated with more side effects than combined mifepristone–misoprostol regimens.	Moderate
Post-2022 evidence on self-managed abortion with accompaniment or remote support.	Observational studies and implementation research.	Self-managed medication abortion with accurate information and access to backup care was generally safe and acceptable, particularly in early pregnancy.	Low to Moderate
Evidence on abortion care among adolescents and young people.	Qualitative evidence, implementation studies, and youth-engagement reports.	Youth-responsive design, confidentiality, respectful care, and meaningful youth engagement improve acceptability and may reduce access barriers.	Low to Moderate
Post-abortion contraception studies after 2022.	RCT follow-up evidence, cohort studies, and systematic reviews.	Immediate post-abortion contraception improves uptake; long-acting reversible contraception is effective when freely chosen, but counselling must avoid coercion.	High for uptake; Moderate for continuation and person-centred outcomes
Health-worker task-sharing evidence after 2022.	Implementation studies and observational evidence.	Trained non-physician providers can safely provide components of abortion and post-abortion care, especially medication abortion and aspiration care within defined protocols.	Moderate

Pain-management evidence after 2022.	RCTs and systematic reviews.	NSAIDs remain first-line for medication abortion pain; multimodal patient-centred pain control improves experience. Evidence remains limited for optimal regimens at later gestations.	Moderate
Legal restriction and access studies after 2022.	Natural experiments, population studies, and health-services research.	Delays, travel burden, and restricted access are associated with later presentation, increased logistical and financial burden, and inequitable access.	Low to Moderate

2.3 GRADE Evidence-to-Recommendation Update Table

Clinical or policy question	New evidence direction after WHO 2022	Benefits	Harms or concerns	GRADE Quality	Recommendation Strength
Should routine ultrasound be required before early medication abortion?	New evidence supports WHO: routine ultrasound is unnecessary for many early abortions when history-based dating and ectopic-risk screening are adequate.	Faster access, lower cost, fewer delays.	Missed ectopic pregnancy risk if screening/follow-up are poor.	Moderate	Strong against routine requirement
Should telemedicine be offered for medication abortion?	Evidence increasingly supports safety, effectiveness, and acceptability.	Improved access, privacy, reduced travel burden.	Requires reliable information, emergency pathways, and safeguarding for coercion or violence.	Moderate	Strong where legally and clinically feasible
Should no-test medication abortion protocols be used?	Post-2022 data support use in selected patients.	Reduced barriers, faster care.	Needs clear eligibility criteria and follow-up guidance.	Moderate	Conditional to Strong depending on system capacity

Should mifepristone–misoprostol remain preferred?	New evidence continues to support superior effectiveness compared with misoprostol alone.	Higher completion, fewer repeat doses.	Mifepristone access may be restricted in some settings.	High	Strong
Should misoprostol-only regimens remain available?	Evidence supports use when mifepristone is unavailable.	Expands access, safe alternative.	Lower effectiveness and more side effects than combined regimen.	Moderate	Strong as alternative
Should self-managed abortion be supported at < 12 weeks?	New evidence supports safety when accurate information and backup care are available.	Autonomy, privacy, access.	Risk if misinformation, coercion, or emergency care barriers exist.	Moderate	Strong with support systems
Should routine in-person follow-up be required after uncomplicated medication abortion?	Evidence supports self-assessment and remote follow-up.	Convenience, reduced burden.	Persistent pregnancy may be missed without clear instructions.	Moderate	Strong against routine requirement
Should immediate post-abortion contraception be offered?	Evidence supports improved uptake when offered voluntarily.	Prevents unintended pregnancy, improves continuity.	Risk of coercive counselling if not rights-based.	High	Strong
Should task sharing be expanded?	Evidence supports trained non-physician provision.	Improves access and system capacity.	Requires training, supervision, referral systems.	Moderate	Strong

Should adolescent-specific engagement be included in guideline and service design?	Youth-engagement evidence supports acceptability and equity.	Improves responsiveness, trust, confidentiality.	Evidence is often qualitative and context-specific.	Low to Moderate	Strong as equity-based good practice
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2.4 Evidence Gaps and Future Research Needs

1. Later-gestation abortion

- More comparative evidence is needed on optimal medication regimens, pain control, cervical preparation, and service models after 12–14 weeks.

2. Pain management

- Evidence remains limited on best multimodal regimens, especially for medication abortion at later gestations and for patients with high pain sensitivity or prior trauma.

3. Adolescent abortion care

- More implementation studies are needed on confidential youth-responsive care, digital support, safeguarding, and outcomes among adolescents.

4. Self-managed abortion

- More prospective evidence is needed on self-management models in legally restrictive, low-resource, and humanitarian settings.

5. Equity and access

- Research should evaluate outcomes by age, race/ethnicity, poverty, rurality, disability, migration status, and exposure to intimate partner violence.

6. Telemedicine

- More evidence is needed on optimal screening, follow-up, language access, privacy protection, and integration with emergency care.

7. Post-abortion contraception

- Studies should prioritize person-centred outcomes, continuation by choice, satisfaction, autonomy, and avoidance of coercion.

8. Implementation under legal restriction

- More health-services research is needed on clinical consequences of delayed care, criminalization, travel requirements, and provider shortages.
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2.5 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
Routine ultrasound should not be required before abortion.	Post-2022 evidence supports history-based gestational dating and ectopic-risk screening for many early abortions.	Maintain. Do not require routine ultrasound; use ultrasound when clinically indicated.
Mifepristone followed by misoprostol is preferred for medication abortion.	New evidence continues to support high effectiveness.	Maintain. Use combined mifepristone–misoprostol where available.
Misoprostol alone may be used when mifepristone is unavailable.	Evidence supports safety and reasonable effectiveness but confirms lower efficacy than combined regimens.	Maintain. Offer misoprostol-only regimens when mifepristone cannot be used or accessed.
Telemedicine may support abortion care.	Large post-guideline observational evidence strengthens confidence in safety and effectiveness.	Strengthen implementation. Telemedicine should be offered where legally permitted and clinically appropriate, with emergency referral pathways.
Self-management at <12 weeks can be supported.	New evidence supports safe self-use with accurate information and backup care.	Maintain and operationalize. Provide clear instructions, warning signs, analgesia advice, and access to follow-up.
Routine in-person follow-up is not needed after uncomplicated abortion.	Evidence supports remote follow-up and self-assessment.	Maintain. Offer optional follow-up rather than mandatory follow-up.
Immediate post-abortion contraception should be offered voluntarily.	Evidence continues to show improved uptake; autonomy concerns remain important.	Maintain. Offer all methods using non-coercive, person-centred counselling.
Task sharing can expand access.	Implementation evidence remains supportive.	Maintain. Train and authorize appropriate health workers with supervision and referral systems.
Adolescents require confidential, non-discriminatory care.	Youth-engagement evidence supports direct involvement of young people in service design.	Strengthen. Build youth-responsive confidential pathways and include young people meaningfully in guideline/service development.
Mandatory waiting periods, third-party authorization, and medically unnecessary restrictions should not be imposed.	Post-2022 access studies show delays and inequities associated with restrictions.	Maintain. Remove medically unnecessary barriers to timely care.

Core diagnostic and eligibility steps

1. Confirm pregnancy.
2. Estimate gestational age.
3. Assess for ectopic pregnancy risk or symptoms.
4. Assess medical contraindications to medication abortion, including:
 - Known allergy to medications.
 - Chronic adrenal failure or long-term systemic corticosteroid therapy for mifepristone use.
 - Bleeding disorders or anticoagulation requiring individualized assessment.
 - Suspected ectopic pregnancy.
5. Discuss options:
 - Medication abortion.
 - Procedural abortion.
 - Continuing pregnancy.
 - Adoption or parenting support where relevant.
6. Provide informed consent and non-directive counselling if desired.
7. Offer pain management and contraception voluntarily.
8. Provide warning signs and emergency contact information.

Red flags requiring urgent evaluation

- Heavy bleeding soaking more than expected or causing dizziness/syncope.
- Severe or worsening abdominal pain.
- Fever or feeling very unwell.
- Foul-smelling discharge.
- Persistent pregnancy symptoms after treatment.
- No bleeding after medication abortion when pregnancy is confirmed.
- Symptoms suggestive of ectopic pregnancy.

References

1. World Health Organization. **Abortion care guideline**. Geneva: World Health Organization; 2022.
2. World Health Organization. **Medical management of abortion**. Geneva: World Health Organization; 2018.
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4. World Health Organization. **Clinical practice handbook for safe abortion**. Geneva: World Health Organization; 2014.
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6. Upadhyay UD, et al. **Effectiveness and safety of telehealth medication abortion in the USA**. *Nature Medicine*. 2024.
7. Aiken ARA, et al. **Effectiveness, safety and acceptability of no-test medical abortion provided by telemedicine**. *BJOG*. 2021.

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9. World Health Organization. **Consolidated guideline on self-care interventions for health.** Geneva: World Health Organization; updated editions.
10. Centers for Disease Control and Prevention. **U.S. Selected Practice Recommendations for Contraceptive Use.** Atlanta: CDC; 2024.